NCCP 00456 · VERSION 4

# Your treatment, explained

**Stage III non-small-cell lung cancer** · NCCP 00456 v4 · Document revised 4 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Cisplatin + etoposide + chest radiotherapy

28-day cycle

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

## When to get help

### Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

### Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, sudden illness, new or worsening breathlessness, troublesome bleeding, inability to drink or swallow, uncontrolled vomiting, very little urine or severe rash. Tell staff at once about pain, swelling, faintness or breathing trouble during an infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

**Open your CUH contact / treatment card**

Sources: nccp · hse-rt · etoposide-smpc

**Choose a topic to read**

YOUR COMPLETE REFERENCE

# Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

- How each drug works
- How the treatments fit together
- Your 28-day timeline
- Baseline and cycle-specific checks
- Whole treatment, scans and checks
- Tablets, visits and practical planning
- Supportive medicines
- Understanding the evidence
- Questions to ask
- Sources

UNDERSTAND YOUR TREATMENT

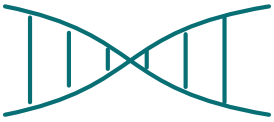
# Chemotherapy and chest radiotherapy

Chemotherapy and chest radiotherapy work together to treat lung cancer. Your team will explain the aim, possible benefit and alternatives for you.

**CHEMOTHERAPY**

### Cisplatin

Platinum chemotherapy



**Damages cancer-cell DNA**

Cisplatin damages the DNA that cancer cells need to divide. It can also affect healthy cells.

DNA damage → Cell repair disrupted → Cell growth reduced

---

**What to watch for**

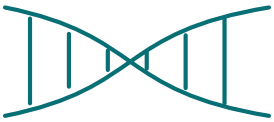
Report fever, unusual bleeding, reduced urine, hearing changes or tingling.

Sources: nccp · cisplatin-smpc

CHEMOTHERAPY

## Etoposide

Topoisomerase II chemotherapy



### Interferes with DNA repair

Etoposide interferes with an enzyme used to copy and repair DNA. This can stop cancer cells growing and can also lower healthy blood counts.

DNA copying → Repair interrupted → Cell growth reduced

### What to watch for

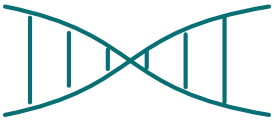
Report fever, bleeding, a sore mouth or feeling faint during an infusion.

Sources: nccp · etoposide-smpc

RADIOTHERAPY

## Chest radiotherapy

Local radiation treatment



### Targets the planned chest area

Radiotherapy directs radiation at the planned chest area. Its appointments and dose are set by the radiation oncology team.

Planned chest area → Targeted radiation → Cancer-cell damage

### What to watch for

Report painful swallowing, poor intake, skin changes, new cough or breathlessness.

Sources: hse-rt

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

**Explore your existing anatomy and side-effect map**

THE BIGGER TREATMENT PLAN

# How the treatments fit together

EACH CYCLE: 28 DAYS

### Two initial cycles with radiotherapy

1 2

Cisplatin on Days 1 and 8; etoposide on Days 1–5. Radiation appointments run alongside chemotherapy as planned by radiation oncology.

ONLY IF PRESCRIBED

### Review before any further cycles

Optional 3 Optional 4

If the cancer has not progressed and treatment remains appropriate, the consultant may prescribe two additional cycles with an agreed radiation plan. These are not automatic.

INDIVIDUAL FOLLOW-UP

### Recovery and next treatment decision

Review

Your team reviews recovery and response, then agrees the next plan. Further drug treatment or surgery is a separate decision.

The team confirms dates, radiation fractions and any optional phase.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

# My tests and checks

Required safety tests are distinct from team-agreed scans and follow-up. Radiotherapy appointments and extra tests follow your personal plan.

| Check | Before treatment | Days 1–5 | Day 8 | Rest of cycle | After / review |
|---|---|---|---|---|---|
| **Chemotherapy** | Confirm the plan | Both Day 1; etoposide Days 2–5 | Cisplatin only | No planned chemotherapy | Review the next phase |
| **Radiotherapy** | Planning and appointments | Starts close to chemotherapy | Follow radiation calendar | Continue if scheduled | Radiation follow-up plan |
| **Blood / organ tests** | FBC, kidneys / liver | FBC before etoposide doses | FBC and symptoms | Extra tests if needed | Recovery / individual scans |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00456 v4 · official regimen · HPRA etoposide product information,14 November2025 · HSE lung cancer treatment: radiotherapy

THE WHOLE TREATMENT JOURNEY

# Cisplatin + etoposide + chest radiotherapy: treatment and checks

## Stage III non-small-cell lung cancer

Cisplatin on Days 1 and 8; etoposide on Days 1–5. Each cycle lasts 28 days. Two cycles first; two more may be offered after review. Radiotherapy usually starts within 24 hours of the first chemotherapy dose; follow your separate radiation appointment plan.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

**Treatment, tests and reviews across the course**

| Care / check | Before treatment | Days 1–5 | Day 8 | Rest of cycle | After / review |
|---|---|---|---|---|---|
| **Chemotherapy** NCCP 00456 · ETOPOSIDE-SMPC | Confirm the plan | Both Day 1; etoposide Days 2–5 | Cisplatin only | No planned chemotherapy | Review the next phase |
| **Radiotherapy** NCCP 00456 · ETOPOSIDE-SMPC | Planning and appointments | Starts close to chemotherapy | Follow radiation calendar | Continue if scheduled | Radiation follow-up plan |
| **Blood / organ tests** NCCP 00456 · ETOPOSIDE-SMPC | FBC, kidneys / liver | FBC before etoposide doses | FBC and symptoms | Extra tests if needed | Recovery / individual scans |

## What each check is for

1

### Prepare both plans

Before treatment

Chemotherapy and chest radiotherapy work together to treat lung cancer. Your team will explain the aim, possible benefit and alternatives for you.

**Ask / act:** Confirm chemotherapy, radiation, tests and contacts.

Sources: NCCP 00456

2

### Review safety

Before each administration

Check bloods, organs and symptoms

**Ask / act:** Report fever, swallowing or breathing changes promptly.

Sources: NCCP 00456 · ETOPOSIDE-SMPC

3

### Review next phase

After the planned cycles

Check response and recovery

**Ask / act:** Get the next plan in writing.

Sources: NCCP 00456

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

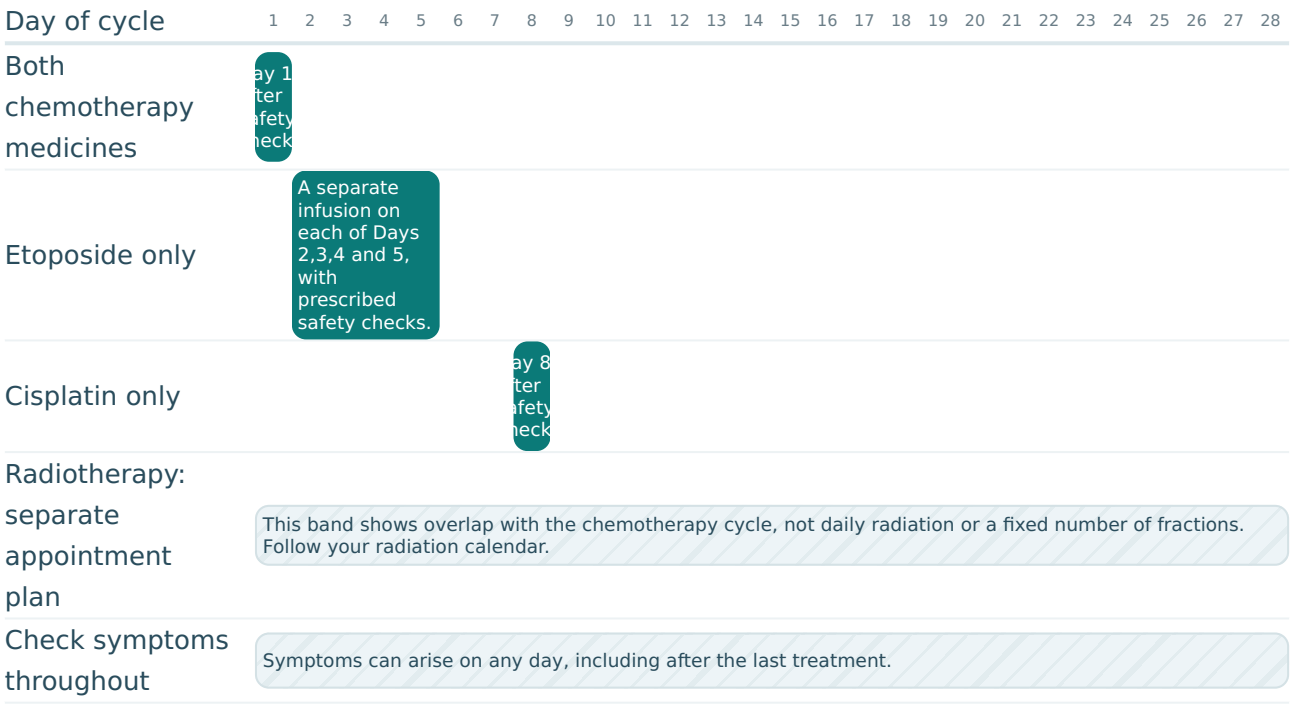
This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.30

# Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Cisplatin on Days 1 and 8; etoposide on Days 1–5. Each cycle lasts 28 days. Two cycles first; two more may be offered after review. Radiotherapy usually starts within 24 hours of the first chemotherapy dose; follow your separate radiation appointment plan.




**Two overlapping plans:** Chemotherapy days do not define the radiation schedule. Follow the written radiation calendar even on a day without chemotherapy.

**Symptoms can build up:** Tiredness and swallowing discomfort may develop during treatment. Breathing changes can also appear later; report them after treatment as well.

**Why these bands stay broad:** Relative chemotherapy days only. Radiotherapy band shows overlap, not a daily dose or fraction count. There is no guaranteed safe day for symptoms. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00456 v4 · official regimen ↗

**There are no guaranteed “safe days”.** Call oncology immediately for fever, sudden illness, new or worsening breathlessness, troublesome bleeding, inability to drink or swallow, uncontrolled vomiting, very little urine or severe rash. Tell staff at once about pain, swelling, faintness or breathing trouble during an infusion.

**Read the whole cycle as text**

### Day 1: Both chemotherapy medicines

Treatment after blood-test and symptom review. Radiotherapy usually begins close to this first treatment. Confirm both treatment plans and take only the prescribed support medicines.

### Day 2–5: Etoposide each day

Return for etoposide on each of these four days. Radiotherapy appointments may also be planned. The team checks blood results and symptoms before dosing. Report swallowing or breathing changes.

### Day 6–7: No planned chemotherapy

Follow the separate radiotherapy calendar. Anti-sickness medicines may continue as prescribed. Call if unwell; do not wait for the next infusion.

### Day 8: Cisplatin only

Return for the second cisplatin dose after safety checks. Extra drip fluids are needed; etoposide is not given today. Confirm today’s anti-sickness plan with the team.

### Day 9–28: No further chemotherapy this cycle

Continue any planned radiotherapy appointments and safety checks. Tiredness and swallowing symptoms can build up. Report new symptoms and keep the urgent contact plan available, including after treatment ends.

PRACTICAL TREATMENT GUIDE

## Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

**Before the first visit**

Confirm both oncology teams, treatment and test appointments, medicines, transport and urgent contacts.

STEP 2

**Frequent visits in week 1**

Plan for chemotherapy on Days 1–5 plus Day 8. Radiation may require other visits; combine calendars with the team.

STEP 3

**After the visits**

Keep support medicines and contact numbers at hand. Ask early for help with swallowing, food, fluids or transport.

**Open practical treatment guide** · **Printable practical guide**

Sources: NCCP 00456 v4 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

# Support around the medicines

### 1 Your anti-sickness plan

Cisplatin days need a different plan from etoposide-only days. The team combines prevention and rescue medicines into one clear schedule. Do not repeat or add courses yourself.

### 2 Eating, drinking and swallowing

Report painful swallowing, weight loss or difficulty drinking early. The team can arrange pain relief, dietitian support or other help.

### 3 Fluids and kidney protection

Follow your individual drinking advice and report much less urine. Cisplatin needs drip fluids before and after dosing; tell staff if you cannot drink or feel swollen or breathless.

### 4 Other medicines and prevention

Bring a full medicine list. Ask before starting medicines, supplements or vaccines. Fertility and contraception advice should be agreed before treatment.

Sources: nccp · ncis · hse-rt

# Understanding the evidence

Chemotherapy and radiotherapy are used together for selected people with Stage III lung cancer.

Published results help guide discussion but do not predict your outcome. The source study was small and did not compare this regimen with another treatment.

Sources: nccp · trial

PREPARE FOR THE CONVERSATION

# Questions worth asking

**Do I attend every day?**

Chemotherapy is planned on Days 1–5 and8 of each cycle. Radiotherapy has a separate appointment calendar; do not use this cycle diagram as the radiation timetable.

**How many cycles will I have?**

Two cycles are planned. Two further cycles may be offered only after review and a new confirmed plan.

**Can side effects continue after treatment?**

Yes. Keep reporting new or worsening symptoms after the last infusion or radiation session. The team agrees your follow-up checks.

> **Try explaining the plan back:** Which chemotherapy days will I attend?,Where is my radiation calendar?,Who will I ring if unwell tonight?

SOURCE TRANSPARENCY

# Where this information comes from

Regimen schedule: NCCP 00456 v4. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00456 v4 · official regimen
- HPRA etoposide product information,14 November2025
- Cisplatin product information
- NCCP NCIS antiemetics V8
- HSE lung cancer treatment: radiotherapy
- SWOG9019 phase II study

---

SACTCheck · Developed by Dr Paul O’Brien · Sources checked 2026-11-02.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

## Written source addresses

https://healthservice.hse.ie/documents/6902/456_V4_CISplatin_50_and_Etoposide.pdf

https://assets.hpra.ie/products/Human/28093/Licence_PA2059-036-001_14112025143401.pdf

https://www2.hse.ie/conditions/lung-cancer/treatment/

https://www.medicines.org.uk/emc/product/6960/smpc

https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf

https://pubmed.ncbi.nlm.nih.gov/12177106/